

Chapter 5

AI in Health Care and Health Disparities



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1 AI in Health Care and Social Work

1.1 *Health and Social Determinants of Health*

Health is widely recognized as a fundamental human right, a principle enshrined in foundational global documents like the Universal Declaration of Human Rights and the Constitution of the World Health Organization (United Nations, 1948). Traditionally, the realization of this right has been equated with health care, the system of diagnosis, treatment, and prevention delivered across settings ranging from acute care hospitals to community clinics. However, despite the universal ideal of health as a right, profound inequities persist, revealing a critical disconnect between the promise of medical care and the reality of population health outcomes (Institute of Medicine, 2008).

This disconnect is most evident in the persistence of health disparities, defined as preventable differences in health outcomes experienced by socially disadvantaged populations. These inequities are not random occurrences; instead, they are

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systematically driven by social, economic, and environmental determinants, such as poverty, structural racism, and unequal access to resources (National Academies of Sciences, Engineering, and Medicine (NASEM), 2017). The consequences are stark: globally, billions lack access to essential health services, and even within wealthy nations like the United States, life expectancy varies dramatically between high- and low-income groups. For instance, a 40-year-old American man in the poorest 1% of the income distribution dies an average of 14.6 years sooner than a man in the wealthiest 1% (Chetty et al., 2016).

To address these disparities, we must look beyond clinical healthcare. While the United States spends far more per capita on medical services than other nations, it spends significantly less on social services, a disproportionate investment that correlates with lagging health indicators compared to peer countries. This suggests that “health” must be defined more broadly than the absence of disease or the presence of medical intervention. It requires a shift in focus toward the social determinants of health (SDOH).

SDOH are the broad socio-economic and environmental factors in which people “are born, grow, work, live, and age” that affect a wide range of health outcomes (CDC, 2024). These include conditions such as income level, housing stability, neighborhood safety, education, employment opportunities, access to nutritious food, transportation, social support, and discrimination. Research indicates that medical care accounts for only a minority of modifiable health outcomes, whereas SDOH influence 80% or more of these outcomes (Magnan, 2017). In other words, SDOH are responsible for most health disparities.

1.2 Social Work and SDOH

Social workers have long been integral to the healthcare system, addressing not only the psychosocial dimensions of health but also the broader social, economic, and structural factors that shape health outcomes, alongside medical care. They work across the continuum of care, including prevention and public health, primary care, acute and specialty care, rehabilitation, home health, long-term care, and hospice. Building on this traditional focus on psychosocial dimensions, contemporary social work practice emphasizes the broader biopsychosocial and spiritual needs of patients. In each setting, social workers attend to biopsychosocial-spiritual needs using a person-in-environment framework, complementing medical professionals’ work by focusing on patients’ social contexts and supports.

A defining feature of healthcare social work is its commitment to health equity (NASW, 2016). Social workers recognize that reducing these health disparities requires addressing both individual biopsychosocial needs and the systemic issues that drive poor outcomes. In other words, healthcare social work operates at both the micro level (e.g., supporting patients and families through illness, treatment, and recovery) and the macro level (e.g., advocating for policies and programs to eliminate inequities in health care). Social workers bring a holistic, justice-oriented

perspective that complements the medical model. This makes them crucial leaders in integrating social care into health care delivery.

Addressing SDOH is imperative for improving population health and reducing disparities. Healthcare organizations are increasingly screening patients for social needs and partnering with community services. Social workers often lead these efforts. For instance, a social worker may implement a brief SDOH screening tool in a primary care clinic to identify patients who are struggling with food insecurity or housing instability. Upon finding a need, the social worker can intervene by connecting the patient to a food pantry, a medical-legal partnership to prevent eviction, or a transportation assistance program. In hospital settings, social workers ensure safe discharge plans by arranging home health services or rehabilitation, recognizing that without stable home supports, a patient's recovery may be compromised. In macro roles, social work administrators advocate for policies, such as expanded Medicaid or paid sick leave laws, that address social determinants at a systemic level. The 2019 National Academies of Sciences, Engineering, and Medicine (NASEM) report, *Integrating Social Care into the Delivery of Health Care*, affirmed that factors like stable housing, economic security, food security, and transportation play a key role in health outcomes (NASEM, 2019). The NASEM consensus study recognized social workers as specialists in identifying and addressing social needs in healthcare settings.

1.3 Current Applications of AI in Health-Focused Social Work Practice

We conducted a scoping review of recent articles from mainstream social work journals to identify studies linking artificial intelligence (AI) with health-related practice. Most of the identified work aligns with three major thematic areas. In the following section, we summarize the papers within each theme and synthesize the insights emerging from this literature.

1.3.1 Mental Health Assessment and Risk Detection

A substantial number of AI-and-social-work studies focus on using AI for mental health screening, diagnosis, or risk prediction. Local LLM architectures can analyze confidential mental health narratives with high fidelity while preserving privacy safeguards (Perron et al., 2025). Studies on youth populations further show that digital traces, such as Reddit or Twitter posts, contain identifiable linguistic patterns of depression, anxiety, and discrimination (Choi et al., 2025; Sinha et al., 2025). Behavioral health research confirms that machine-learning models can detect suicidal ideation and early deterioration signals more rapidly than routine screening methods (Ballı et al., 2025; Ji et al., 2021). Some studies also indicate mixed

readiness to integrate these systems, often expressing concerns about explainability and the clinical consequences of potential AI errors (Creswell Báez et al., 2025). These findings underscore AI's growing capacity to enhance mental health risk identification while also highlighting the ethical imperatives, especially around fairness and contextual interpretation, that must accompany such applications.

1.3.2 Child Welfare, Parenting Support, and Family Health

Child and family services are among the most active areas of AI experimentation in the social work field. Large language models have demonstrated the ability to classify complex child-protective case reports with accuracy comparable to trained humans, enabling scalable synthesis of abuse narratives (Stoll et al., 2025). Efforts to evaluate the reliability of AI judgment in interpreting children's statements further illustrate both the promise and sensitivity of these tools (Karataş, 2025). Machine-learning studies examining resilience and caregiving environments offer new insights into family-system factors in childcare and occupational health (Duan et al., 2025). AI-enabled parenting support platforms have the potential to enhance parental skills and support early intervention through tailored guidance (Escoredo et al., 2025). These developments resonate with broader child-welfare scholarship documenting how predictive models can help with the early detection of maltreatment risks while simultaneously raising concerns about bias, surveillance, and equity. Overall, these studies demonstrate both the promise and tensions of AI in child and family systems, positioning technology as a potential enhancer of safety and support, so long as its implementation remains grounded in social work values and ethical safeguards.

1.3.3 Communication, Social Media, and Online Behavior Monitoring

AI-driven analysis of communication and digital behavior has become an important methodological direction for understanding youth well-being, risk behavior, and social environments. NLP models can identify emotional expressions, discrimination experiences, and mental health signals in youth digital communication (Choi et al., 2025; Zhang et al., 2025). AI-supported interpretations of sexting, sextortion, and broader online sexual-risk patterns provide additional avenues for digital-safety intervention (Pater et al., 2025). Parental engagement and youth-safety applications also leverage online communication streams to detect risk and provide structured support (Escoredo et al., 2025). These developments mirror broader research showing that AI can detect cyberbullying, self-harm indicators, and social isolation patterns in adolescents' digital footprints (Milosevic et al., 2022). At the same time, the literature emphasizes the ethical challenges of monitoring minors' online behavior, including privacy expectations, consent boundaries, and potential disparate impacts on marginalized youth.

1.4 Current Applications of AI in Healthcare

AI is transforming nearly all sectors of health and medicine, including but not limited to higher-quality disease diagnosis, personalized treatment, drug discovery, public health forecasts, mental health support, remote patient monitoring, patient self-care, and healthcare administrative processes (Alowais et al., 2023; Bajwa et al., 2021). In this section, we briefly highlight several major areas where AI has had the most significant impact.

1.4.1 Diagnosis

One of the most mature applications of AI in healthcare is disease diagnosis. AI has the potential to accelerate and improve disease diagnosis by assisting clinicians in interpreting diverse patient data, including imaging, lab results, and clinical notes (Niazi et al., 2019). A well-known example is a deep neural network trained on over 129,000 skin lesion images, which achieved a classification accuracy equivalent to that of board-certified dermatologists in discriminating malignant melanomas from benign lesions (Esteva et al., 2017). AI systems are also being implemented to assist in detecting abnormalities such as lung nodules on chest X-rays, tumors on CT/MRI, and breast cancer on mammograms (Khalifa & Albadawy, 2024; McKinney et al., 2020). There is also growing use of AI in laboratory testing and clinical microbiology. ML models trained on genomic and metagenomic data can detect pathogens, predict antibiotic susceptibility, and improve workflow efficiency, thereby increasing the speed and accuracy of infectious disease diagnosis (Peiffer-Smadja et al., 2020; Vandenberg et al., 2020).

1.4.2 Treatment Planning

Clinicians often need to consider a range of relevant variables when developing a treatment plan, including the patient's medical history, disease stage, comorbidities, and preferences. AI tools could serve as decision-support tools by employing algorithms to process and evaluate relevant variables and recommend evidence-based strategies (Alowais et al., 2023). In the example of sepsis management, it was reported that the treatment selected by the AI model is, on average, reliably higher than that of human clinicians, and patients had the lowest mortality when both the human physician and the AI clinician recommended the strategy (Komorowski et al., 2018). AI has significantly improved treatment planning for cancer radiotherapy, a traditionally complex, time-intensive process that aims to deliver sufficient radiation to the tumor while preserving healthy tissue (Wang et al., 2019).

1.4.3 Drug Discovery

AI is widely regarded as a powerful accelerator and enhancer of drug discovery and development. One well-known example is the deep learning–based identification of a new antibiotic compound, later named Halicin (Trafton, 2020). Another landmark advance is AlphaFold, an AI system developed by DeepMind that accurately predicts protein folding; in 2021, it released predicted structures for almost all human proteins (Jumper et al., 2021). AlphaFold now serves as an increasingly valuable structural biology resource, helping drive drug development. AI can also contribute through drug repurposing—the process of identifying new indications for existing drugs (Pushpakom et al., 2019). Instead of focusing solely on novel compounds, AI can uncover new therapeutic uses by recognizing patterns in large, complex datasets (Cortial et al., 2024; Yan et al., 2024).

1.4.4 Precision Medicine

Precision medicine, sometimes known as “personalized medicine”, is an innovative approach to tailoring disease prevention and treatment that considers differences in people’s genes, environments, and lifestyles (Ashley, 2016). The intersection of AI and precision medicine presents “remarkable opportunities” to identify disease at early stages or give patients the right therapy they would most likely benefit from (Matheny et al., 2019). AI approaches are applied to genomic, clinical, electronic health records, medical imaging, and other “omics” (e.g., proteomics, metabolomics) datasets to identify phenotypes or predictors that guide more individualized therapeutic pathways (Johnson et al., 2021). For example, AI systems can analyze tumor genomic sequences to identify genetic mutations or expression patterns to guide targeted therapies (El-Deiry et al., 2019; Huang et al., 2018).

1.4.5 Predictive Analytics and Population Health

In healthcare, predictive analytics are used to forecast infectious disease outbreaks, identify patients at high risk for certain conditions, and inform preventive care and policy interventions. Predictive models driven by AI could evaluate patient demographics, clinical history, comorbidities, and social determinants of health to identify higher-risk individuals for chronic disease management and/or specific complications and hospitalizations (Dixon et al., 2024). For instance, AI systems have demonstrated remarkable efficacy in predicting the occurrence of specific cancers and cardiovascular diseases (Singh et al., 2024), as well as hospitalization and hospital readmission risk (Riis et al., 2023; Silva et al., 2024). AI is also employed to estimate population health trends and outbreaks by integrating different data sources such as weather, travel patterns, health surveillance data, and social media to quickly identify emerging infectious disease threats and alert public health officials (Ding et al., 2025).

1.5 Harnessing AI to Address SDOH

AI has emerged as a powerful tool for identifying and addressing SDOH, with the potential to reduce global health disparities. AI could help identify and monitor SDOH. Machine learning and predictive analytics integrate data from electronic health records, public health surveillance, and socioeconomic and environmental datasets to uncover hidden patterns linked to SDOH (Basu et al., 2020). For example, AI models have linked food insecurity and substandard housing to higher chronic disease rates, and flagged pollution-heavy areas with elevated asthma risk. AI is also being used in public health surveillance to forecast disease outbreaks and map social vulnerabilities. Natural language processing and advanced language models can mine unstructured text for SDOH, vastly extending the reach of social needs identification beyond structured records (Guevara et al., 2024).

AI could also help with intervention design and policymaking. AI-generated insights enable health systems and governments to implement targeted interventions on upstream factors. Predictive risk scores incorporating SDOH help clinicians proactively identify vulnerable patients and connect them with resources like housing or food assistance. At the community level, AI can pinpoint which neighborhoods would benefit most from new services, guiding more equitable allocation of resources. Nonetheless, using AI for SDOH demands careful ethical oversight. Ensuring diverse, representative data and mitigating algorithmic bias are essential so that these tools help mitigate, rather than exacerbate, disparities.

2 AI for Social Work Across Healthcare Settings

The Social Work Health Impact Model illustrates how social workers can act across multiple domains of intervention: clinical practice, prevention and health promotion, systems alignment, and broader policy advocacy (Cederbaum et al., 2018). Against this backdrop, AI technologies could play a transformative role across these domains. Whether through predictive analytics in healthcare settings, natural language processing for case documentation, or AI-enabled tools that support advocacy with big data evidence, AI is becoming intertwined with the mission of healthcare social work. This section discusses social work practice in various healthcare settings, highlighting the unique contributions of social workers and the implications of AI in each setting.

2.1 Prevention and Public Health

In prevention and public health, social workers focus on community-level and population-level interventions to improve health outcomes and reduce risks before illness occurs. This can include roles in public health departments, community health centers, schools, and nonprofit agencies that address issues such as vaccination, obesity, HIV prevention, substance abuse, and more. Social workers bring to these efforts an understanding of community dynamics, engagement strategies, and policy advocacy, making them natural leaders for prevention initiatives that require both social and medical strategies.

AI is revolutionizing this work by enabling “Precision Public Health”, i.e., the use of data to target interventions more effectively to those most in need (Cordeiro, 2024). Rather than treating health solely as a medical issue, AI tools allow social workers to analyze complex interactions between social environments and health outcomes. AI can help identify communities or subpopulations with elevated health risks. Machine learning models combining data on SDOH indicators can predict which neighborhoods might experience spikes in issues like asthma exacerbations or opioid overdoses. A public health social worker could use this information to allocate resources proactively, such as organizing an asthma awareness and home-cleanup drive in an area with high asthma-related emergency department visits. Social workers leveraging AI-driven platforms can enhance outreach and education. AI systems can scan data, including unconventional sources such as Google search trends and social media mentions, for early signs of public health issues. A well-known example is using Google search query patterns to detect flu outbreaks faster than traditional reporting (Yang et al., 2015).

Prevention is an area where global collaboration often occurs. Tools like AI-driven GIS mapping help identify communities globally that lack access to clean water or basic healthcare, guiding interventions. Social workers’ intersectional lens ensures that prevention programs consider factors like gender, race, and socioeconomic status, which AI data can incorporate if well-designed. For example, in a vaccination campaign, AI might optimize the locations of vaccine sites (Musa et al., 2025). Still, a social worker will ensure those sites are accessible and welcoming to minority communities that historically faced discrimination in healthcare.

Crucially, social workers bring an intersectional lens to these technologies. While AI might optimize logistical resource distribution, such as mapping the most efficient locations for vaccine sites, social workers ensure these locations are culturally accessible and trusted by marginalized communities. By integrating AI’s analytical power with social work’s community engagement strategies, the profession can advance a holistic vision of health that addresses the root causes of disparity.

2.2 *Primary Care, Acute Care, and Specialty Care*

Primary, acute, and specialty care social work is dynamic and directly patient-facing. Across all settings, social workers address both clinical and SDOH to improve patient outcomes and care continuity. The social worker's challenge and opportunity are to use these tools to enhance patient care, such as faster identification of needs and more personalized interventions, while also humanizing the care, making sure technology does not eclipse empathy and individualized support.

Many hospitals have adopted AI predictive models, such as risk scores for readmission, falls, and complications (González-Castro et al., 2024). These models often trigger automatic consults or alerts to social work. For instance, a readmission risk model might flag a patient with heart failure who lives alone and has multiple comorbidities as high risk. This means social workers can focus on patients who most need attention rather than purely on those who happen to ask or be referred late.

Primary and specialty care have embraced telehealth, especially post-2020. AI often underpins telehealth platforms, such as routing patients to providers, summarizing patient input for providers, and translating language (Chaturvedi et al., 2025). Social workers providing tele-behavioral health might use AI tools for scheduling, even for elements of therapy, such as tracking progress. These augmentations can improve continuity but also risk turning social workers into data monitors, so balance is needed.

2.3 *Rehabilitation*

Social workers in rehab facilities perform many familiar functions, but in the context of patients dealing with significant life changes. They provide psychosocial support to patients adjusting to new physical limitations or chronic disability, addressing grief, identity issues, family strain, and motivation for the hard work of rehab. They educate patients and families about coping strategies and available long-term resources (e.g., vocational rehab, disability benefits, and home modifications). Discharge planning is a major task, as patients often cannot return to their previous living situation without modifications or help. Social workers coordinate with insurance and community agencies to arrange durable medical equipment, home care, and follow-up services. They might run peer support groups, for example, connecting new spinal cord injury patients with others who have lived with such injuries. They are also key in family education, preparing family caregivers for the patient's needs and linking them to support like respite care or caregiver training programs.

In rehabilitation, perhaps more than anywhere else, the interplay between mindset and physical progress is strong. AI can support physical training and planning, but *mindset* is the domain of human support. As one might say, “rehab is 10% physical, 90% mental.” Social workers ensure that while PTs and OTs work on the body,

someone's caring for the soul. AI won't provide a patient with empathetic listening or the therapeutic alliance that helps them find meaning after a life-altering injury, which remains the sacred work of the social worker. Suppose AI offloads paperwork or finds the best accessible housing option faster. In that case, the social worker has more time to sit with a patient who is mourning the loss of their former abilities and help them envisage a fulfilling life ahead. That is the real "value-add" that technology should enable in rehab social work.

2.4 Long-Term Care

In long-term care (LTC) settings, social workers perform psychosocial assessments upon admission and periodically thereafter, develop care plans addressing mood, behavior, and social adjustment, provide or coordinate counseling to cope with aging, loss, and transition, and lead group programs. They are problem-solvers across a range of issues, from roommate conflicts to residents refusing care to family dynamics in decision-making. Social workers also address quality-of-life domains, e.g., helping residents maintain autonomy, engage in meaningful activities, and stay connected to their community or family. They often handle end-of-life planning and advocate for residents' preferences during care plan meetings. Ensuring culturally competent and person-centered care is a core function.

Falls are a big concern in LTC. AI-powered camera systems or wearable sensors that detect falls or analyze gait patterns to predict fall risk are being introduced in some facilities (Subramaniam et al., 2022). For example, there are camera systems that can alert staff if a resident gets out of bed at night. Social workers might not operate these systems directly, but they change their practice environment by potentially reducing falls and raising privacy considerations. Many LTC residents have dementia or cognitive impairments and may exhibit behavioral symptoms. AI can help track patterns of agitation, such as through movement trackers or by analyzing vocalizations via microphones to detect stress indicators, to address triggers preemptively.

Social workers are often liaisons to families. Tech is now bridging families and residents, for example, with secure messaging apps for updates and video call set-ups for families who can't visit. AI could assist by automatically sending families updates (e.g., "Your mom participated in bingo today and had a good appetite at lunch"), thereby relieving family anxiety. Also, AI sentiment analysis could gauge family satisfaction from messages or survey responses, flagging those needing follow-up (Taherdoost & Madanchian, 2023). A social worker could receive a nudge, such as "the daughter of Resident X expressed concern about care quality in her last message," so they can reach out promptly instead of waiting for a quarterly survey result.

3 AI for Social Work Across Healthcare Practice

Healthcare social workers fulfill a variety of roles and responsibilities that can be grouped into key functional areas: screening and assessment, care planning, service delivery and intervention, advocacy, quality evaluation, and record-keeping. These correspond to core practice standards for social work in health settings (NASW, 2016). In each of these activity domains, AI and related technologies are beginning to play a transformative role. This section explores how AI can be applied in healthcare social work practice.

3.1 *Screening and Assessment*

Screening and assessment are often the first steps in social work practice with clients. In healthcare settings, this involves gathering a psychosocial history, identifying patient needs (practical, emotional, and social), assessing risks (such as risk of abuse, suicide, readmission, etc.), and determining eligibility for various services. Social workers also conduct SDOH screenings, asking questions about factors like housing, food, transportation, and social support (Gehlert & Browne, 2011; NASEM, 2019).

AI is increasingly augmenting these processes. Machine learning algorithms can analyze EHR data and other patient information to predict who is at high risk of certain adverse outcomes. NLP algorithms can digest large volumes of unstructured text, such as clinical notes, social worker case notes, or even social media posts, to surface insights for assessment. In healthcare, AI could be used in collaborative documentation in behavioral health to evaluate the quality of person-centered care. For example, in child welfare, predictive analytic tools ingest case narratives and structured data to estimate the likelihood of child maltreatment recurrence, aiding social workers in screening high-risk cases. While these tools are still emerging, they point to a future where AI can comb through data that would be impossible for an individual to read, giving social workers a “heads up” about potential issues.

Some health systems have begun deploying chatbot-style questionnaires accessible on tablets or phones that ask patients about social needs in a friendly, conversational manner. These AI-driven screening tools can increase the rate of disclosure of sensitive needs by providing a private, stigma-free interface. For instance, a patient may feel more comfortable typing into a tablet that they have trouble affording food than telling a clinician face-to-face, especially if the chatbot normalizes these questions for all patients. The social worker then receives an alert or report on any needs identified. This doesn’t replace the social worker’s comprehensive assessment, but it makes the process more efficient by front-loading information gathering.

3.2 *Care Planning*

Care planning in social work involves crafting a tailored plan with the client to address identified needs and goals. In healthcare settings, a social work care plan might include linkages to community services, therapeutic interventions, education, and coordination with the medical team's treatment plan. It is often a collaborative process: social workers partner with clients to set goals, and with other professionals to align services.

AI can assist care planning in several ways, mainly by providing decision support and information management. A common challenge for social workers is keeping track of the ever-changing landscape of community resources. AI-enabled resource directory platforms now exist that can suggest optimal referrals based on a client's specific profile. For instance, if a client is a low-income older adult living alone with diabetes, the system might automatically suggest a nearby Meals on Wheels program, a transportation service for medical appointments, and a diabetes self-management class. Some platforms incorporate AI search algorithms that learn from past referrals and outcomes, which resources tend to be most effective. Rather than thumbing through binders or static databases, the social worker can enter key parameters to receive a curated list of resources. This is especially helpful when working with issues like housing, where multiple programs have different criteria; the AI can quickly filter them. It effectively functions as a specialized "recommender system" for social care services.

Beyond concrete services, AI can parse research and aggregate client data to recommend intervention strategies. For example, consider a hospital social worker planning discharge for a patient with congestive heart failure and limited family support. An AI tool might analyze similar past cases of patients with heart failure living alone, with certain risk factors, and suggest a set of interventions that led to better outcomes, such as enrolling the patient in a remote monitoring program, scheduling a home health nurse visit within 48 h of discharge, and ensuring a social work follow-up call within a week. It might even prioritize which factor (e.g., transportation, medication management) to address first using predictive modeling. Another scenario is in mental health care planning: an AI might identify that a client with moderate depression and social isolation could benefit most from a combination of a peer support group and a digital CBT app, in addition to therapy. This combination could reduce depressive symptoms significantly. These kinds of insights can guide social workers in crafting more effective, evidence-informed plans faster than manually sifting through the literature or relying on anecdotal memory.

It's important to stress that the human element remains central in care planning. AI can provide options or even optimal pathways based on generalized data. However, the social worker and client must decide together what truly fits the client's preferences, culture, and unique situation (NASW, 2017). When thoughtfully integrated, AI can significantly enhance care planning. It can reduce the clerical load and provide data-driven insights, allowing social workers to spend more time in conversation with clients. The social worker remains the driver steering the course in partnership with the client.

3.3 *Service Delivery and Systems Integration*

Social workers do not only assess and plan; they also deliver interventions and coordinate action. In healthcare, this can range from brief counseling and psychoeducation with a patient, to arranging concrete services, to leading care coordination meetings, to macro-level interventions like community health initiatives. The NASEM (2019) report provides a framework for action that classifies health and social care integration activities into five categories: Awareness, Adjustment, Assistance, Alignment, and Advocacy. Here, we discuss how AI tools could be applied across the five categories.

Awareness refers to activities to identify social risks and needs. AI tools can increase awareness by systematically identifying needs. Whether it's through patient-level SDOH screening chatbots or analysis of community-level data, AI expands the scope and speed of identifying where intervention is needed. For example, an AI might analyze demographic, socioeconomic, and claims data to produce a “heat map” of a city showing neighborhoods with the highest preventable hospitalization rates and correlated social issues. A health system's social work department could use this to guide community health workers to those areas.

Adjustment refers to making changes in clinical care to accommodate patients' social barriers. AI can help inform care design adjustments. For instance, using machine learning to analyze appointment data, a clinic might discover, via an algorithm, that a significant number of patients from a certain low-income ZIP code miss morning appointments. In response, the clinic extends hours into evenings or offers telehealth for that community. AI could also assist on the fly: a scheduling system might automatically suggest a telehealth option to a patient who lives far from the clinic. Additionally, AI-driven translation tools can help adjust communication for patients with limited English proficiency, enabling more linguistically appropriate service delivery.

Assistance refers to connecting patients directly with social resources and support, often through referrals or navigation. When providing direct aid, such as connecting a client to resources, AI can streamline referral and navigation tasks. Voice-activated assistants might walk patients through application forms for social programs. Social services agencies could use robotic process automation to quickly determine eligibility for benefits, reducing wait times. Some hospitals have experimented with AI-based texting services that “assist” patients post-discharge, for example, by checking whether they have obtained their medications or need help, and then alerting a social worker if an issue arises. These intelligent follow-ups increase the reach of assistance beyond the clinic visit. Importantly, because social needs often require quick action, AI's ability to operate 24/7 is a boon for timely aid, e.g., a chatbot that provides emergency shelter addresses after hours.

Alignment refers to aligning healthcare with community resources by partnering with organizations, investing in community capacity, or otherwise strengthening the resource landscape. Social workers in leadership may use AI analytics to align programs with community needs. For instance, if an AI analysis finds that many

diabetes patients are food-insecure and there's no healthy food program in the area, a social work administrator might establish a partnership with a food nonprofit (alignment) or start a hospital-based produce prescription program. Alignment often involves forming intersectoral partnerships. Here, AI's contribution might even be in matchmaking: platforms can identify potential community partners by mining data on local organizations to recommend where a health system should invest or collaborate to strengthen the social care infrastructure. The caveat is that smaller community organizations might not have robust data for AI to analyze, so personal relationship-building remains key.

Advocacy involves promoting policies and broader system changes to address SDOH and improve community health equity. AI empowers social workers' advocacy by providing robust evidence and narratives. Big data can illustrate problems that were previously hidden. For example, analyzing millions of health records might reveal that a particular policy led thousands of vulnerable patients to skip care. Visualizing this data makes a compelling case to policymakers. Without oversight, algorithms can increase racial bias in care, but with proper use, data can also uncover bias and spur reform. The aforementioned case of the racially biased health algorithm is a prime example. Once exposed, it not only prompted the company to fix the tool but also fueled broader advocacy around algorithmic fairness in health-care. Social workers have joined coalitions calling for algorithmic accountability in public services to protect clients from unjust outcomes. Internally, social work leaders advocate within health systems for ethical guardrails on AI, emphasizing core values like equity, transparency, and client privacy. NASW itself has called for an AI Commission to ensure social work has a voice in shaping AI's impact on our field and those we serve. On a community level, AI data can support classic social work advocacy for resources: e.g., open data on housing and neighborhood conditions, when analyzed, can "empower communities to fight disinvestment and predatory lending" by pinpointing patterns of neglect and harm.

3.4 Quality Improvement and Learning Health Systems

Continuous practice evaluation and quality improvement are essential in healthcare social work. Social workers strive to evaluate the outcomes of their interventions, measure program performance, and use data to improve services. In the era of big data and AI, social workers have powerful new means to conduct these evaluations and contribute to a learning health system. AI enhances the learning loop in healthcare social work. It enables more rigorous, rapid analysis of the effectiveness of interventions, supports a data-driven culture of improvement, and helps quantify the value that social work adds to healthcare outcomes. As healthcare moves toward value-based care, demonstrating and improving quality is crucial. Social workers, armed with AI insights, can both improve services and solidify their role as key players in achieving the "Triple Aim" of better care, improved population health, and lower costs (NASW, 2017). In practice, that could mean showing that adding a

social needs screening and intervention program not only improves patient satisfaction but also reduces costly hospital utilization for vulnerable groups, making a strong case for sustainability.

On a larger scale, social workers contribute to practice-based research. AI is accelerating social work research by enabling the analysis of “big data” from health systems, government databases, and even social media relevant to community health. For example, researchers have merged large administrative datasets (child welfare, social services, and public health) to study how policy changes affect outcomes. Social work scholars are using AI to examine how large social datasets correlate with health measures, potentially identifying novel community-level indicators. The generation of new knowledge through such interdisciplinary research feeds back into practice improvement. The findings from AI analyses can highlight best practices and model programs that can be replicated.

4 Ethical Challenges and Role of Social Workers

While AI holds great potential to augment social work practice in health care, integrating AI introduces significant ethical hurdles, particularly regarding algorithmic bias, which can perpetuate the very health disparities that social workers strive to eliminate (Ferrara, 2024; Hanna et al., 2025). AI systems learn from historical data, and thus they risk absorbing and amplifying societal biases, potentially leading to discriminatory outcomes in which care is unequally allocated based on race or socioeconomic status. Social workers are uniquely positioned to advocate for equity by demanding that AI development include diverse, representative data and undergo rigorous bias testing.

Recognizing this, critical thinking has emerged as an essential social work competency in the age of AI. Practitioners must actively interrogate algorithmic outputs, questioning what might be missing or misrepresented rather than accepting recommendations at face value. Experts caution that overreliance on AI can erode professional judgment and empathy, making it vital for social workers to maintain human oversight and advanced critical thinking when using these tools. Indeed, social workers will require extremely high levels of critical thinking to evaluate and critique AI systems effectively and to spot biases, epistemic gaps, or cultural misinterpretations in algorithmic information. To this end, the profession must invest in structured AI-related training and digital literacy education. Social workers should be trained to recognize AI’s limitations and to challenge algorithmic outputs when necessary, and recent scholarship calls for integrating AI literacy, i.e., knowledge and skills to understand, use, and critically evaluate AI, into core social work training competencies. Such preparation will enable practitioners to engage with AI tools in an equitable, person-centered manner while mitigating potential harms.

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